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ScienceDirect

Behavior Therapy 55 (2024) 1189–1204

Behavior Therapy

www.elsevier.com/locate/bt

State of the Science: The Unified Protocol for Transdiagnostic Treatment of Emotional Disorders

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Before the development of the Unified Protocol for the Transdiagnostic Treatment of Emotional Disorders (UP), evidence-based treatment options for commonly co-occurring anxiety, mood, and related disorders consisted of numerous single-disorder protocols that shared many similarities, reflecting the overlap among these disorders themselves. The UP distilled common elements of cognitive behavioral protocols into a unified intervention directly targeting core temperamental features underlying these disorders of emotion, namely neuroticism and associated emotion dysregulation. The UP has since become a leading "transdiagnostic" treatment for emotional disorders, which is now available in several formats (e.g., individual, group, digital) and has accumulated a strong evidence base, leading to international implementation. There is now also research evidence that the UP can be flexibly applied to a range of clinical presentations, including borderline personality disorder, trauma- and stressor-related disorders, eating disorders, alcohol use disorder, and comorbid chronic physical health conditions. Yet additional research is needed to evaluate the UP in routine clinical settings, with more heterogeneous patient populations, and under circumstances that mirror actual clinical practice. Thus, we must also continue to explore the benefits of large-scale UP training initiatives and implementation in major healthcare systems.

Keywords: emotional disorders; transdiagnostic; unified protocol; cognitive-behavioral treatment; neuroticism

Dr. Barlow receives royalties for books and treatment manuals including the *Unified Protocol* from Oxford University Press, Guilford Publications Inc., Cengage Learning, and Pearson Publishing. Grant monies for various projects come from the National Institute of Mental Health, the National Institute of Alcohol and Alcohol Abuse, and Colciencias (Government of Columbia Initiative for Science, Technology, and Health Innovation). Consulting and honoraria during the past several years have come from the Agency for Healthcare Research and Quality, the Foundation for Informed Medical Decision Making, the Department of Defense, the Renfrew Center, the Chinese University of Hong Kong, Universidad Católica de Santa Maria (Arequipa, Peru), New Zealand Psychological Association, Hebrew University of Jerusalem, Mayo Clinic, and various American Universities. Dr Farchione receives royalties from Oxford University Press for the *Unified Protocol for the Transdiagnostic Treatment of Emotional Disorders* and research funding related to the reported work from the National Institute of Mental Health and Juvenile Diabetes Research Foundation. He also received consulting (personal) fees from the MacArthur Foundation, the National Center of Neurology and Psychiatry in Japan, Brightside Health, Tage S.R.L., Allegheny Health Network, and Grupo Pacifico outside the submitted work.
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THE UNIFIED PROTOCOL for Transdiagnostic Treatment of Emotional Disorders (UP) had its origins in a book published in 1988, *Anxiety and Its Disorders*, and a subsequent article, "Disorders of Emotion" (Barlow, 1988, 1991). Based on the accumulated experience of several decades of treating and researching anxiety and related disorders, much of it occurring in one of the first specialty clinics devoted to that purpose, the Center for Stress and Anxiety Disorders at SUNY Albany initiated in 1981, it seemed to us that, in a general

sense, the treatments we were implementing for the variety of anxiety disorders had much in common. Specifically, despite very disparate symptomatic presentations ranging from obsessive-compulsive disorder (OCD) to specific phobias and posttraumatic stress disorder (PTSD), all intervention strategies involved provoking anxiety either in imagination or in vivo (exposure) and preventing attempts at escape and avoidance while at the same time attending to the accompanying "hot" apprehensive cognitions consisting of maladaptive attributions and appraisals. Most treatments in those days also incorporated hypothetical arousal reduction techniques such as relaxation, breathing retraining, etc., which were later demonstrated, for the most part, to be noncontributory to outcomes. In a chapter in Barlow (1988) titled "The Process of Fear and Anxiety Reduction: Affective Therapy," we first outlined a therapeutic approach for the full range of anxiety (and depressive) disorders based on the ancient tenets of emotion theory dating back to Darwin (1872). For it was Darwin who first described "action tendencies" associated with strong emotions, such as the intense urge to escape a dangerous situation triggered by the emotion of fear, a phenomenon we now refer to as "emotion-driven behaviors." Of course, these action tendencies are highly adaptive in normal circumstances but also sustain a maladaptive emotional response if present. These action tendencies became one of the first transdiagnostic targets for change in that early transdiagnostic conception. Other core targets included a pervasive sense of unpredictability and uncontrollability over life stressors present in these disorders. As our theorizing evolved, these ideas were extended to the full range of disorders characterized principally by emotion dysregulation, which we began to refer to as "emotional disorders" (Barlow, 1991). In addition, we came to consider the sense of uncontrollability so prominent in these presentations to be part of the core underlying temperament of neuroticism that drives emotional disorders (Barlow, Ellard, et al., 2014).

Those early ideas were put aside for over a decade while we focused on further developing and evaluating "single-diagnosis" interventions such as treatments for panic disorder in large clinical trials (e.g., Barlow et al., 2000). In those years, the radical transformation in diagnosis and classification appearing for the first time in DSM-III (APA, 1980) that transitioned our nomenclature from very general unreliable concepts such as "neurosis" to more specific operationally defined and reliable categories of psychopathology was still new. This innovation clearly sparked considerable research on what came to be known as evidence-based interventions, both psychological and pharmacological. As these very thin slices of psychopathology could be operationalized and measured, newly developed interventions were also operationalized, and their effects became more targeted and apparent. Although we have moved beyond that conception (see below), this was an essential and necessary step in the subsequent development of "transdiagnostic" treatments.

In 2004, we revived our focus on targeting shared features of emotional disorders with the publication of an article "Toward a Unified Treatment for Emotional Disorders" (Barlow et al., 2004). At that time, recognizing the plethora of treatment manuals for each specific anxiety, mood, and related disorder reflecting DSM-IV categories, we returned to the approach first articulated in 1988: identifying a common set of principles of change applicable to all these disorders. At the same time, research on the classification and nature of emotional disorders conducted with our colleague Tim Brown underscored how fundamental temperamental aspects underlying anxiety, mood, and related disorders seemed more central to these disorders than did the symptom presentations that were the defining features in DSM-IV (and DSM-5) systems (e.g., Brown & Barlow, 2009). This, in turn, led to a greater focus on the underlying temperament of neuroticism, as well as related traits and temperaments such as extraversion or positive affect, and the beginnings of conceptualizations of treating these temperaments directly rather than focusing on disorder-specific symptoms, as described in more detail below. The protocol that emerged consists of five core therapeutic procedures thought to be *transdiagnostic*, or widely applicable to all disorders of emotion. The remainder of this state-of-the-science review (see Comer, 2024) provides the rationale for this approach; describes the UP as it now exists; and lays out recent theoretical developments, supporting evidence, innovative applications from around the world, and recent progress on dissemination and implementation.

A Functional Model of Emotional Disorders

We have conceptualized a range of conditions, including anxiety, obsessive-compulsive, and depressive disorders, as "emotional disorders" (Bullis et al., 2019). Such disorders are characterized by frequent, intense, and excessive negative emotions that are experienced as aversive (see

Bullis et al., 2019; Barlow et al., 2021; Sauer-Zavala & Barlow, 2021). A functional definition of emotional disorders conceives of negative emotionality and aversive reactivity to emotional experience as dimensional constructs associated with genetic and learned vulnerabilities and labeled neuroticism (Barlow et al., 2021; see Figure 1).

Neuroticism, in turn, mediates the risk for the development of emotional disorders. Specifically, a sense of limited control and unhelpful beliefs about one's ability to tolerate or influence events and outcomes, including the emotional experience itself, leads to avoidant cognitive or behavioral coping to manage emotional discomfort. This emotion-motivated, avoidant coping may relieve emotional discomfort in the short term. However, this process can, paradoxically, increase subsequent emotional experiences and/or make intense emotions feel even less tolerable. This leads to a greater tendency to avoid or escape these experiences and reinforces catastrophic beliefs about emotions, thus maintaining emotional disorders (Barlow et al., 2021; Bullis et al., 2019; Sauer-Zavala & Barlow, 2021). While triggers for negative emotions and avoidant coping strategies may differ based on specific DSM disorders (e.g., social performance situations for social anxiety disorder; internal and external cues associated with past trauma for PTSD), neuroticism is viewed as the core transdiagnostic mechanism that facilitates the development of pathological symptoms following salient emotional experiences as illustrated in Figure 1.

In the functional model described above, heightened negative emotionality and aversive reactivity to emotional experiences or neuroticism emerges as the primary vulnerability for anxiety and depressive disorders (Barlow, 2000; Barlow et al., 2021; Brown, 2007; Brown & Barlow, 2009). Brown (2007) found that dimensions of neuroticism could explain most of the temporal covariance among DSM-5 anxiety and depressive disorders. That is, the expression and severity of specific symptoms (i.e., somatic, cognitive, or interpersonal), in particular DSM-5 emotional disorders, are incidental and of secondary importance to the temperamental similarities in predicting the course of these disorders over time (Barlow, Ellard, et al., 2014). It follows that neuroticism also significantly contributes to comorbidity among psychological disorders. For example, in one study, neuroticism accounted for 20-45% of such comorbidity (Kessler et al., 2011).

Triple vulnerability theory (Barlow, 2002) was developed to explain the etiology of emotional disorders, including the emergence of neuroticism itself (Sauer-Zavala & Barlow, 2021). Under this theory, heritable traits (generalized biological vulnerability), along with a sense of unpredictability and uncontrollability derived from early experiences (generalized psychological vulnerability), are dynamic diatheses underpinning neuroticism (Brown & Barlow, 2009). A third specific psychological vulnerability accounts for the emergence of one emotional disorder instead of another (Sauer-Zavala & Barlow, 2021).

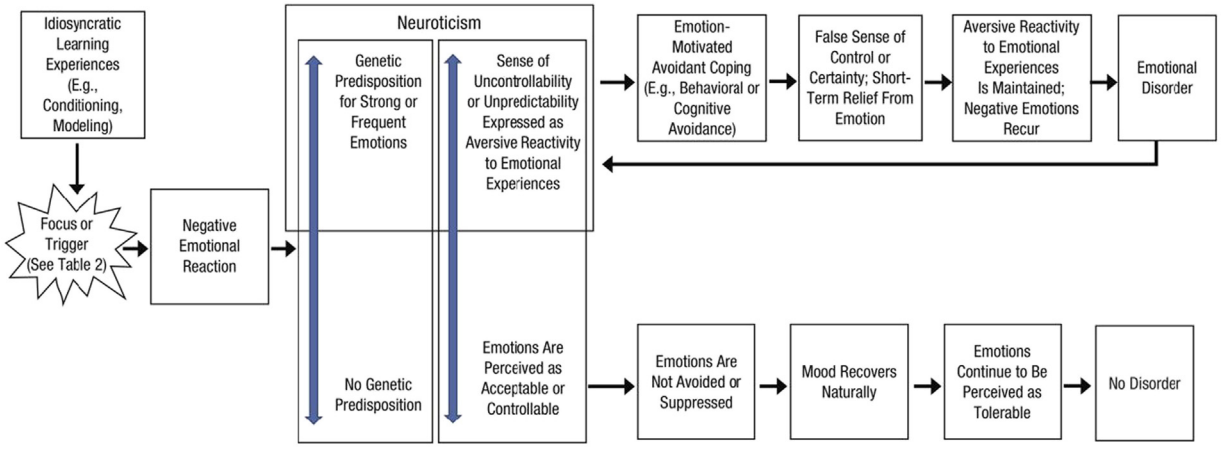

FIGURE 1 Model of the mechanisms underlying the development of emotional disorders compared to normal emotional experiences (from Barlow et al., 2021). Emotional triggers generate negative emotional reactions for most individuals, but the consequences differ depending on the person's level of neuroticism. Individuals higher in neuroticism, who have developed a sensitivity to certain triggers from learning experiences during early life, respond in ways that lead to the development and maintenance of emotional disorders (top pathway). Individuals with lower neuroticism tend to respond more adaptively to negative emotions, resulting in healthy emotional functioning (bottom pathway).

The biological vulnerability to certain personality traits is widely evidenced, with genetic factors explaining 40-60% of the variance in the temperamental trait of neuroticism (Bouchard & Loehlin, 2001; Kendler et al., 2003). Genetic contributions are particularly important in younger individuals who have fewer environmental stressors (Laceulle et al., 2013). Therefore, preliminary evidence suggests that the tendency to react strongly to negative stimuli, often referred to as hyperexcitability of limbic structures, and the difficulty returning to baseline emotionality present in those with neurotic temperament, are genetically mediated.

However, these strong reactions alone do not fully explain the development of neuroticism. Instead, neuroticism develops when the tendency to respond to negative stimuli with excess excitability is combined with acquired perceptions of unpredictability and uncontrollability (Chorpita & Barlow, 1998). Animal and human studies have demonstrated that a sense of unpredictability or uncontrollability is strongly implicated in developing trait neuroticism and stress resilience (Sauer-Zavala & Barlow, 2021). Thus, early environmental experiences, such as chronic stress and trauma, foster an underlying sense of perceived uncontrollability in vulnerable individuals, which produces enhanced negative emotionality, increasing the saliency of environmental events (White et al., 2006). As such, the relationship between the inherited negative emotionality and environmental factors is bidirectional and dynamic. It can be said that the generalized biological (genetic) diathesis influenced by generalized psychological diatheses to view events as unpredictable and uncontrollable produces a psychological disorder (e.g., panic disorder, OCD), the form of which is influenced by early learning experiences reflecting a specific psychological vulnerability (Barlow, 2002; Barlow, Ellard, et al., 2014; Sauer-Zavala & Barlow, 2021).

The Unified Protocol

Before the development of the UP, various single-disorder protocols had been developed and empirically tested for the treatment of individual emotional disorders, which unsurprisingly shared many of the same procedures as noted above. While these interventions were shown to be efficacious, the preponderance of manuals presented a problem for dissemination due to the high training burden and significant cost to therapists and healthcare systems. The UP was developed as an attempt to distill the common elements among these protocols into a coherent, unified intervention applicable to the full range of anxiety and related disorders.

The UP is a transdiagnostic, cognitive behavioral intervention that targets temperamental characteristics, particularly neuroticism and resulting emotion dysregulation, present across emotional disorders (Barlow, Farchione, Sauer-Zavala, et al., 2017). It includes eight treatment modules typically delivered over 12 to 16 treatment sessions. These modules build upon each other and are designed to be delivered sequentially. The initial modules of the UP set the groundwork for emotional regulation skills training. The first module focuses on building motivation and setting goals for treatment. It is followed by a module providing psychoeducation on the nature of emotional disorders and the function of emotions. These are succeeded by five "core modules" focusing on emotion regulation strategies and their application during exposure to emotional experiences, namely: (1) mindful emotional awareness, (2) cognitive flexibility, (3) countering emotional behaviors, (4) recognizing and confronting physical sensations, and (5) emotion exposure. The final module focuses on consolidating treatment and consists of reflecting on progress, skills review, and relapse prevention. The outcomes data reviewed below are based on the sequential administration of these modules. Yet it is worth noting at this point the results from a preliminary study by Sauer-Zavala et al. (2019), suggesting that it may be feasible to reorder core treatment modules according to assessed pretreatment skill level in each individual patient and that delivering modules to target specific individual skill weaknesses may lead to earlier treatment improvements.

Outcomes Data

Results from the initial randomized clinical equivalence trial comparing the efficacy of the UP to well-established evidence-based single-disorder CBTs (SDPs) in a large sample of 223 patients with principal anxiety disorders (i.e., generalized anxiety disorder, social anxiety disorder, OCD, and panic disorder with/without agoraphobia) found equivalent reductions in symptoms of these emotional disorders among the UP and SDPs that were maintained during the 6-month follow-up, though treatment with the UP resulted in less attrition (Barlow, Farchione, Bullis, et al., 2017). Results from subsequent studies support the maintenance of these improvements at 12 months (Eustis et al., 2020) and (in a smaller sample of treatment completers, N = 80) up to 36 months following treatment (Bullis et al., 2023). Many additional studies have evaluated the treatment efficacy of

the UP across a range of internalizing disorders; meta-analytic reviews have reported large effect size symptom reductions across anxiety, depressive, and related disorders, including borderline personality disorder, following the UP (Carlucci et al., 2021; Cassiello-Robbins et al., 2020; Longley & Gleiser, 2023; Sakiris & Berle, 2019). Notably, Cassiello-Robbins et al. (2020), using PRISMA guidelines, reviewed 77 studies across a wide range of problems including anxiety, depressive, bipolar, traumatic-stressor, eating, borderline personality, insomnia, and physical health disorders, as well as nondiagnosable problems such as nonsuicidal self-injury and sexual minority stress. Results indicated that the UP can be flexibly applied to various diagnostic presentations. In addition, some of these analyses suggest that the UP not only demonstrates superiority to waitlist controls and treatment as usual for reducing neuroticism/negative affect but also shows modestly greater efficacy for all anxiety disorders compared to traditional CBT, with comparable reductions in depression symptoms (Longley & Gleiser, 2023). Further, the UP is associated with decreased use of maladaptive emotion regulation (ER) strategies and increased use of adaptive ER strategies, consistent with its treatment targets (Sakiris & Berle, 2019).

Effects on Neuroticism

The triple vulnerability theory described above specifies neuroticism as the core temperamental feature across disorders of emotion responsible for symptomatic expression and the principal target of the UP. Accordingly, and contrary to the supposed "stability" associated with the term "temperament," the UP was designed to target this trait directly (Sauer Zavala & Barlow, 2021), and recent research supports reductions in neuroticism following treatment (e.g., Sauer Zavala et al., 2021). More recently, Stumpp, Southward, and Sauer-Zavala (2023) demonstrated that it was changes in neuroticism during treatment with the UP that predicted symptom change as opposed to symptom change leading to changes in neuroticism, and that state-level changes in neuroticism occurred by Session 6 whereas trait-level changes were evident by the end of treatment. These results suggest that successfully targeting neuroticism may constitute the fundamental mechanism of action in transdiagnostic treatments such as the UP.

Novel Treatment Delivery Formats

The UP was designed for individual delivery but has now been adapted to group and digital formats. These formats build on the efficiency of UP by improving accessibility while also reducing wait times, cost, and therapist burden (Ramos et al., 2024). The group adaptation of the UP generally consists of twelve 2-hour sessions incorporating homework review, new material, practice exercises, and homework assignments (Bullis et al., 2015). UP group treatment has demonstrated comparable efficacy to group CBT for anxiety disorders (Peris-Baquero & Osma, 2023) and to pharmacotherapy only (i.e., an individualized medication plan) in terms of improvements in symptoms of anxiety, depression, and well-being (de Ornelas Maia et al., 2015). In a large noninferiority RCT from Denmark, 291 patients with anxiety or depressive disorders in primary care settings were assigned to either a group UP intervention or to single diagnosis groups receiving disorder specific interventions. The results demonstrated noninferiority of the UP to the individual protocols, both of which produced significant symptom reduction and increases in well-being (Reinholt et al., 2020).

Digital adaptations of the UP also have several advantages, including the potential to reduce geographic and help-seeking stigma-related barriers. Several web-based versions of the UP have been independently developed and evaluated with various levels of therapist support, demonstrating promising results. The first of these versions was evaluated in a randomized-controlled trial (RCT) utilizing a Romanian sample of adults (N = 105) with anxiety and/or depression (Tulbure et al., 2018). Participants engaged in 9 sessions covering the core modules of the UP and received asynchronous support from a therapist consisting of written communications. Results reflected medium to large improvements in anxiety and depression symptoms, superior to the waitlist control (Tulbure et al., 2018). A second, large RCT with over 200 participants with emotional disorders in Spain utilized a digital adaptation of the UP ("EmotionRegulation") supplemented with a few additional DBT skills (González-Robles et al., 2020). EmotionRegulation consists of 12 self-directed modules supported by phone calls from therapists and twice-weekly automated text message reminders to engage. That intervention demonstrated significant reductions in depression and anxiety as well as increased quality of life compared to treatment as usual, with results maintained at a 3-month follow-up (Gonzalez-Robles et al., 2020). Finally, a RCT in Germany evaluated a web-based version of the UP with 129 adults with anxiety, depressive, and somatic symptom disorders, incorporating 10 sessions that were therapist-guided (Schaeuffele et al., 2022). Results

demonstrated more significant improvements in depression, anxiety, quality of life, and positive and negative affect in the UP condition compared to a waitlist control.

The UP has also been applied in varied clinical settings, including psychiatric inpatient units and primary care. Individual therapy with the UP demonstrated feasibility and acceptability for a small sample (N = 12) of acute, high-risk individuals reporting a recent suicide attempt or active suicidal ideation who were hospitalized in a community crisis stabilization unit (Bentley et al., 2017). Subsequently, a modified version of the UP in group format was implemented within a hospital-based inpatient unit for suicidal thoughts/behaviors and affective disorders patients (Bentley et al., 2020). This adaptation consisted of daily emotion awareness, emotion exposure, and review/practice groups, as well as motivation enhancement and emotion regulation skills groups (e.g., cognitive flexibility, emotional behaviors) offered every other day. Therefore, all modules were delivered in the first two days of admission. Patients who received the UP (N = 133) demonstrated significant improvements in depression, anxiety, and emotion regulation skills compared to baseline, demonstrating the promise of the UP in offering a consistent framework for inpatient care of patients with multiple comorbidities.

Finally, an abbreviated version of the UP in group format (consisting of five 90-minute sessions covering modules related to emotional awareness, cognitive flexibility, emotional behaviors, and relapse prevention) was piloted in the Primary Care Mental Health Integration program within the Veterans Health Administration healthcare system (De Paul & Caver, 2021). Participants (N = 48) generally met the criteria for a single depressive, anxiety, or trauma/stressor disorder. They demonstrated significant improvements in depression, anxiety, and functional impairment (with many falling in the subclinical range posttreatment), underscoring the potential efficacy of brief transdiagnostic interventions with primary care patients (De Paul & Caver, 2021).

Applications to Children and Adolescents

While the UP was developed for use with adults, a version for use with children and another for adolescents (UP-C/A; Ehrenreich-May & Kennedy, 2021) have also been developed, evaluated, and found to be efficacious in randomized clinical trials (e.g., Ehrenreich-May et al., 2017; Kennedy et al., 2019). The UP-C/A has been adapted to specific clinical populations, including children exhibiting serious mental illness (Weintraub et al., 2020), irritability (Hawks et al., 2020), and trauma resulting from natural disasters (Goetz et al., 2023), among other applications (see Ehrenreich-May & Kennedy, 2021). Alternative settings in which the UP-C/A has been applied include partial hospitalization programs (Kennedy et al., 2022). Adaptations for different cultures have also been implemented, including a version for adolescents in Iran (Mohajerin et al., 2023; Zemestani et al., 2023). The UP-C/A has produced its own literature incorporating the considerations relevant to interventions with children and adolescents, including the unique expression of particular disorders in this population (see Ehrenreich-May & Kennedy, 2021).

Applications to Specific Clinical Populations

Although the purpose of the UP is to apply cognitive-behavioral techniques across a range of disorders that share mechanistic commonalities, several contributions have been made that explicate the conceptualization of particular primary diagnostic presentations applied under this common framework, along with evidence of efficacy.

PRIMARY DEPRESSIVE DISORDERS

In evaluating the efficacy of the UP with primary depressive disorders using a subset of 44 participants with comorbid depressive and anxiety disorders from the large equivalency trial of the UP (Barlow, Farchione, Bullis, et al., 2017), Sauer-Zavala et al. (2020) found that depressive symptom clinical severity improved from pre- to posttreatment, and continued improvement was observed at a 12-month follow-up, with additional improvements in associated suicidal ideation. Further, several studies have demonstrated that UP is associated with improvement for patients with primary depressive disorders, including case studies (Boswell et al., 2014; Boswell & Bugatti, 2016; Crespo-Delgado et al., 2020; Farchione et al., 2017; Osma et al., 2018), as well as larger clinical trials in Iran (Bameshgi et al., 2019) and Brazil (Maia et al., 2013). In these case studies and trials, improvements were noted in depressive and anxiety symptoms, as well as quality of life, underscoring the efficacy of the UP for individuals with primary depression. Enhanced treatment results were also evident when the UP was integrated into existing interventions for bipolar disorder with comorbid anxiety disorders. In a small RCT with 29 participants, the group receiving the UP plus pharmacological treatment as usual (TAU) evidenced significantly reduced anxiety and depression compared to TAU (Ellard et al. 2017).

BORDERLINE PERSONALITY DISORDER

Beyond the spectrum of anxiety and unipolar mood disorders, the UP has been applied more broadly to other disorders conceived as emotional disorders. For example, the UP was applied to borderline personality disorder along with comorbid anxiety or mood disorders in a clinical replication series consisting of five cases. The results demonstrated that treatment with the UP was well received by the participants, and there were reductions in borderline personality disorder symptomatology (with large effects) along with depressive and anxiety disorders (with medium effects; Sauer-Zavala et al., 2016). Similar results were reported by Lopez and colleagues (2015, 2019) utilizing a single-case experimental design.

TRAUMA- AND STRESSOR-RELATED DISORDERS

Further, the UP has been applied to trauma- and stressor-related disorders. In a major clinical trial described in more detail below (Castro-Camacho et al., 2023), a contextual and cultural adaptation of the UP (CXA-UP) was proven effective in the treatment of PTSD and comorbid emotional disorders for 200 individuals exposed to armed conflict during the 60-year-long war recently concluded in Colombia. Also, using a subset of 69 participants from a sequential multi-assignment randomized trial of the UP, Hood et al. (2021) found that approximately half of the participants with PTSD showed improvement in clinical PTSD severity, as well as on measures of anxiety, depression, and distress aversion. Finally, Gutner et al. (2022) conducted a hybrid-1 effectiveness/pre-implementation study randomizing 37 male and female trauma-exposed veterans with one or more emotional disorder diagnoses to the UP, present-centered therapy, or TAU. Only the UP exhibited significant decreases in self-reported anxiety and depression as well as decreases in comorbid emotional disorders.

EATING DISORDERS

Reflecting a major effort to extend the application of the UP to eating disorders, a modified version of the UP was evaluated in a large implementation trial ($N = 616$) within a residential eating disorders facility (Thompson-Brenner, Boswell et al., 2018; Thompson-Brenner, Brooks, et al., 2018). The UP was chosen for sitewide implementation by a private, national provider of services for severe eating disorders in an attempt to adopt a consistent treatment model. While several empirically supported treatments for eating disorders were considered, these were perceived to have limited application for residential settings (in which eating behavior was highly regulated) and failed to address high rates of severe, comorbid emotional disorders within their patient population (Thompson-Brenner, Boswell, et al., 2018; Thompson-Brenner, Brooks, et al., 2018). The UP's focus on emotion regulation was viewed as most appropriate, given that negative affectivity/emotional lability, emotional avoidance, and negative beliefs about emotion are known to contribute to the development and maintenance of eating disorders. The resulting adaptation of the UP demonstrated efficacy for reducing symptoms of eating and comorbid emotional disorders among this sample of 616 women compared to the existing treatment regimen in this nationally known eating disorder center, with results maintained at a 6-month follow-up (Thompson-Brenner, Boswell et al., 2018).

PROLONGED GRIEF DISORDER

The conceptual theory underpinning the UP has also been extended to the newest diagnosis in the DSM-5-TR, Prolonged Grief Disorder (PGD; Spencer-Laitt, Curreri, et al., 2023a), a condition where symptoms of grief are prolonged and exacerbated rather than diminishing with time (Jordan & Litz, 2014; Shear, 2015; Shear et al., 2013). The theoretical argument that PGD is an emotional disorder suggests the promise of transdiagnostic approaches such as the UP for treating PGD and its comorbidities as an alternative to well-established, evidence-based single-disorder protocols (Shear et al., 2014, 2016). A small pilot of a 12-week group adaptation of the UP for PGD and comorbid disorders, delivered over Zoom, is currently underway at Boston University's Center for Anxiety and Related Disorders.

ALCOHOL USE DISORDER

Finally, the UP has demonstrated efficacy in treating emotional disorders comorbid with alcohol use disorder (Ciraulo et al., 2013). A large randomized controlled trial ($N = 75$) recently evaluated the efficacy of the UP compared to an active psychoeducational control ("Take Control" for alcohol use disorder; Farchione et al., 2021). Initial findings reflect that participants in both treatment groups showed reductions in weekly average drinks and percentage of drinking days between baseline and posttreatment. However, only participants who received the UP showed a significant decrease in their percentage of heavy drinking days. Furthermore, while participants in the UP group began with a higher percentage of heavy drinking days at baseline than those in the Take Control group,

they showed significantly lower levels of alcohol consumption at post-treatment in terms of weekly average drinks and percent drinking days.

Applications to Health Conditions

Individuals with chronic physical disease show higher rates of psychopathology than the general public, and emotional disorders are the most prevalent form of psychopathology among those facing long-term physical health conditions (Naylor et al., 2012). Therefore, the UP has shown promising clinical utility when used to address a wide range of physical health complaints (see Osma & Farchione, 2023). For instance, an RCT of the UP in the treatment of 64 patients with irritable bowel syndrome (IBS) found that the UP resulted in improvements in IBS symptoms, as well as greater reductions in depression, anxiety, and stress compared to a waitlist control (Mohsenabadi et al., 2018). In addition, evidence from two RCTs with multiple sclerosis patients (resulting in a combined total of 134 participants) indicated that compared to TAU, UP group treatment was associated with more significant reductions in symptoms of depression, anxiety, worry, and emotion dysregulation, as well as more significant improvements in positive and negative affect, that were maintained across 3-month follow-up (Nazari, Aligholipour, et al., 2020; Nazari, Sadeghi, et al., 2020). Clinically significant symptoms of anxiety and depression are also a common feature of Parkinson's disease, with prevalence rates as high as 26% and 35%, respectively (Broen et al., 2016), and these symptoms may have neurobiological origins resulting from dopaminergic system degeneration (Kano et al., 2011). Case studies and single-case experimental designs have provided preliminary support for the feasibility, acceptability, and clinical utility of delivering the UP for Parkinson's patients, with participants showing improvements in anxiety or depressive symptoms, or both, after 12 weekly sessions, changes that were generally maintained at 6-week follow-up (Piers et al., 2022; Reynolds et al., 2020). Finally, type 1 diabetes poses significant psychosocial challenges to youth and can take an emotional toll on parents, which may negatively impact children's metabolic control (Butwicka et al., 2016; Nieuwesteeg et al., 2016). In addition, young adults with type 1 diabetes have a higher prevalence of depressive and anxiety disorders, which can significantly interfere with diabetes self-management during a period of transition toward increased independence (Bryden et al., 2001; Young-Hyman et al., 2016). The UP is well equipped to address these problems by targeting emotion dysregulation directly and can readily integrate health behavioral management strategies relevant to type 1 diabetes. A small study (N = 16) conducted in Iran found that when children with type 1 diabetes and their mothers each received transdiagnostic treatments integrating UP skills (n = 8), they showed improvement in anxiety and emotion regulation compared to TAU (n = 8; Ghasemzadeh & Naghsh, 2020). Building on these findings, an RCT is underway in the U.S. to test a telemedicine-delivered, UP-based transdiagnostic, cognitive behavioral therapy protocol (UP-CBT), enhanced with continuous glucose monitoring to target anxiety and depressive mood disorders in young adults with type 1 diabetes (clinicaltrials.gov Identifier: NCT05000021).

In addition, the UP has provided the foundation for brief interventions for both cancer patients and survivors. Piloting a preventative intervention with 36 breast cancer patients at risk for depression provided initial evidence of its feasibility and associated improvements in depressive symptoms and emotion regulation skills (Weihs et al., 2019). This intervention consisted of four sessions (two 45-minute phone sessions and two in-person sessions lasting 2 hours) along with daily exercises delivered over 6 weeks (Weihs et al., 2019). Also, CanCope, an online intervention for cancer survivors consisting of four core UP modules (i.e., emotional awareness, mindfulness, cognitive flexibility, and countering emotional behaviors), has demonstrated high feasibility and acceptability in a series of pilot studies (Smith et al., 2022). The pilot studies examining the effects of the individual modules indicated that they each promote significant improvement in their targeted skill along with reductions in anxiety. In addition, the emotional awareness and cognitive reappraisal modules were associated with decreased depressive symptoms (Smith et al., 2022). Given these promising results, future RCTs should aim to test the efficacy of the intervention in its entirety.

Applications to Marginalized Populations

The UP has also been adapted to target problems related to minority stress among gay and bisexual men, which are associated with symptoms of depression, anxiety, and co-occurring health risks (Pachankis, 2014). UP modules were adjusted to include identifying minority distress experiences, addressing avoidance reactions to minority stress (e.g., substance use, condomless sexual activity), attributing distress to minority stress as opposed to personal failings, and practicing assertiveness skills as an adaptive response to minority stress

(Pachankis et al., 2015). Ten weekly sessions of this intervention, Effective Skills to Empower Effective Men (ESTEEM), led to significant improvements in depression, alcohol use, and risky sexual behaviors (e.g., sexual compulsivity, casual sex, condomless sex with casual partners) among 63 gay and bisexual men compared to waitlist control, results that were generally maintained at follow-up (Pachankis et al., 2015). A small pilot test of ESTEEM for 13 HIV-positive gay and bisexual men with high levels of sexual compulsivity also resulted in significant improvements in depression and anxiety symptoms. However, challenges related to session attendance were noted (Parsons et al., 2017). Moderate reductions in sexual compulsivity and emotion regulation difficulties were also observed but did not reach statistical significance (Parsons et al., 2017). While the results from these studies are promising, more extensive clinical trials are needed to detect changes in minority stress-related processes (e.g., sexual orientation stress, rejection sensitivity, internalized homophobia) thought to underlie mental health disparities. Future research should continue these efforts to enhance LGBTQ-affirmative care (Burger & Pachankis, 2024) by examining the effectiveness of ESTEEM among other sexual minority groups, while also employing stronger comparison groups.

Homeless individuals also show higher rates of mental disorders than the general population but have less access to evidence-based treatments (Burt et al., 2016; Gutwinski et al., 2021). The UP's transdiagnostic approach to treatment may be uniquely suited to address the substantial rates of psychological disorder comorbidity within this population and reduce the training burden on the often underresourced clinicians who serve them (Sauer-Zavala et al., 2019). A clinical trial of twelve 90-minute sessions of the group UP for homeless women (N = 53) was conducted in Madrid, Spain (Marín et al., 2021). The results reflected high patient satisfaction and perceived usefulness, good group cohesion, and acceptable attendance despite patients' complex life circumstances (Marín et al., 2021). Furthermore, a pilot study of an abbreviated 5-module adaptation of the UP for homeless individuals (N = 6) delivered in a community clinic found high acceptability rates for patients and providers. However, there were notable challenges to administration, including daily survival difficulties faced by patients, high rates of interpersonal conflict and health problems, variability in reading comprehension, attention difficulties, substance misuse, and high provider turnover (Sauer-Zavala et al., 2019). While the findings from this research are promising, larger-scale studies utilizing control conditions will be necessary to determine the effectiveness of these interventions and address challenges to implementation (Marín et al., 2021).

Cross-Cultural Applications

The UP has been subject to empirical scrutiny in diverse global contexts, some of which were reviewed above, with encouraging results affirming its efficacy. The research spans multiple regions, encompassing Europe, with a focus on Spain and the United Kingdom; Asia, particularly Hong Kong and Japan; South America, covering Brazil and Colombia; and Scandinavia, particularly Sweden and Denmark, in addition to Canada (Cassiello-Robbins et al., 2020).

A recent study in Spain examined the effectiveness of UP in a group format within the public mental health system (Osma et al., 2022). Four hundred eighty-eight participants were randomly assigned to receive either the UP or disorder-specific cognitive behavior therapy. Findings demonstrated significant improvement in both conditions, with the UP group exhibiting more significant positive effects on depression, anxiety, and quality of life. Consistent with other studies, these positive outcomes were sustained at follow-up.

In Hong Kong, a recent pilot study involving Chinese adults conducted in community settings with a group format also showed notable effectiveness (Powell et al., 2021). The study, which included 31 participants with anxiety and depression, found that a substantial percentage of participants no longer met criteria for depression (45%) and anxiety (29%) at posttreatment. This suggests the potential effectiveness of the adapted-UP version in Chinese settings, with comparable effect sizes to those observed in Western populations.

Castro-Camacho and colleagues adapted the original UP to target the specific needs of those individuals exposed to the particular context of continuous violence and adverse living conditions of the armed conflict in Colombia as briefly noted above (CXA-UP; Castro-Camacho, Moreno, et al., 2019; Castro-Camacho, Rattner, et al., 2019). Following intervention refinement in consultation with the community, 200 internally displaced persons were assigned to either the treatment group, receiving 12 to 14 twice-a-week or weekly individual sessions, or a waitlist control (Castro-Camacho et al., 2023). Unique to this context was the very high prevalence of kidnapping as the primary stressor in this population and the fact that danger and threat persisted in many cases since the kidnappers remained at large living

among the victims. Results indicated significant reductions in anxiety, depression, somatic and posttraumatic stress symptoms, as well as improvements in quality of life from baseline to posttreatment compared to the control, which were maintained at 3-month follow-up.

To enhance the global accessibility of the UP, efforts have been made to translate both the therapist guide and patient workbook into various languages, considering diverse cultural perspectives and group formats. These translations include Spanish, with regional variations tailored for Spain and Colombia, as well as German, Polish, Italian, Dutch, Swedish, Finnish, Danish, Simplified Chinese, Japanese, and Korean.

Positive Traits and Emotions

When addressing symptoms of emotional disorders in treatment, targeting deficits in negative emotion regulation is one way to promote improvement. However, a complementary emphasis on fostering positive emotions can also support broader well-being. Increasingly, the field is coming to acknowledge that mental wellness is not simply the absence of mental illness. Although related, they are nevertheless distinct aspects of complete mental health. Our secondary analysis of results from the initial equivalence trial comparing the efficacy of the UP to single-disorder CBTs reflects this in several ways. While the findings indicated that both anxiety symptoms and broader well-being (encompassing emotional, psychological, and social components) improved significantly during these treatments, their patterns of change were unique, with the majority of change in well-being occurring during the second half of treatment following the initial decline in anxiety symptoms (Gallagher et al., 2020). In addition, positive affect improved during treatment, and these gains were associated with reductions in both anxiety and depression symptoms (Spencer-Laitt, Long, Woodard, et al., 2023b). Positive expectations in the form of hopeful thinking also generally increased, and this improvement was also associated with decreased anxiety symptoms (Gallagher et al., 2019). When we examined long-term follow-up data, we found that gains in well-being were generally maintained up to 3 years after treatment, and posttreatment levels of hope were predictive of well-being at the final 36-month follow-up time point. In contrast, residual symptoms of anxiety or depression were not predictors of well-being (Long et al., 2023). This suggests that incorporating interventions focused on positive traits and emotions may lead to even more broad-based improvement in mental health.

Accordingly, when we converted the original UP to a digital format delivered through an online platform, we also developed and tested an augmented version—the UP+—that more directly targeted positive affect within a sample of 120 patients with anxiety and depressive disorders. Initial findings from this clinical trial (clinicaltrials.gov Identifier: NCT05120232) indicate that savoring, or the intentional upregulation of positive affect, improved in both versions and was associated with lower levels of anxiety and depression symptoms (Moreno et al., 2023). Furthermore, greater positive affect was directly related to aspects of psychological flourishing, including happiness and life satisfaction, character and virtue, and meaning and purpose, but also indirectly through increased gratitude (Spencer-Laitt, Long, Boschulte, et al., 2023). While the strategies targeting positive affect included in our enhanced version of the UP did not show specific effects in this RCT, the broad improvements in flourishing observed in both versions were encouraging. As such, we plan to explore ways to target and enhance other character virtues that promote resilience. It is our hope that attending to broader well-being during treatment and intentionally fostering positive functioning will lead to more lasting improvements in mental health.

Future Directions

Since the first edition of the UP was published in 2011 (Barlow et al., 2011), research has consistently supported the UP as a treatment for a wide range of anxiety and related disorders with strong emotional components. The UP, with its focus on cross-cutting temperamental factors related to neuroticism rather than surface-level DSM-based symptoms, has now been successfully adapted across many different clinical settings and applications around the world, as reviewed above. With solid efficacy data from numerous clinical trials in place, many investigators have turned their attention to effective and efficient implementation methods. Some of these efforts have been touched on above, such as the large implementation trial within a nationally known residential eating disorders facility (Thompson-Brenner, Boswell et al., 2018; Thompson-Brenner, Brooks, et al., 2018). Using a "train the trainer" model, 335 clinicians were trained at the primary residential site, and enhanced outcomes were sustained over a 6-month follow-up. Subsequently, training has been extended to 1,815 clinicians across all residential and outpatient sites nationwide. Established quality control mechanisms with a continued focus on training, given naturally occurring turnover in per-

sonnel, suggest the successful sustainability of this program.

Nevertheless, more substantial efforts to scale up evidence-based practices including the UP will be necessary to reach a larger segment of the population in need (McHugh & Barlow, 2012). These efforts have been supported by the recently established Unified Protocol Institute (UPI), an organization focused on training mental health care providers and treatment teams to implement the UP in clinical practice settings (Unified Protocol Institute, 2024). The UPI continues to train practitioners worldwide by providing workshops, clinical consultation, and supplementary training materials. In addition, the UPI provides treatment adaptation and implementation support. Recent examples of larger scale initiatives include a five-session version of the UP in group format that has been deployed throughout the Department of Defense, a two-week intensive program for veterans at an intensive outpatient treatment program, and partnerships with clinical social work and psychology doctoral programs across the country to implement evidence-based training curriculums based on the Unified Protocol. The UPI has established partnerships with other UP experts internationally to expand the reach and impact of the UP on a global scale, including in Russia, Denmark, Norway, Japan, Portugal, South Africa, and Spain.

Further, in partnership with Psychwire, a leading provider of online courses and training for mental health professionals, we developed a comprehensive UP training course. This self-directed course spans 6 weeks with expert support and more than 12 hours of video lessons. The course provides didactic instruction, clinical demonstrations, and additional resources such as research papers and homework forms.

As another step towards expanding access to evidence-based care, several ongoing research efforts are focused on developing and refining internet-based applications of the UP. In one example from our group, an English-language, digital version of the UP delivered with asynchronous weekly written therapist support was recently developed in collaboration with SilverCloud Health by Amwell (SCH), a leading digital mental health company. The effectiveness of this program is now being evaluated within the Irish public health primary care system (Timulak et al., 2022). Specifically, a parallel-group, randomized controlled trial comparing the internet-based UP with an enhanced waitlist control is being conducted within a Dublin-based primary care psychology service (part of the Health Service Executive in Ireland), with a planned sample of 120 patients. If successful, the iUP may emerge as a valuable addition to Ireland's public mental health services, potentially fitting seamlessly within a stepped-care model and complementing existing forms of treatment. Further, we are also evaluating the digital UP within the Kaiser Permanente (KP) primary care system as part of a large NIMH-funded pragmatic trial (clinicaltrials.gov Identifier: NCT05567640) that aims to advance machine learning algorithms and develop empirically derived precision treatment rules to enhance the decision-making process in selecting the most effective digital program for individual users. In this trial, 1800 KP members referred by mental health providers as part of routine care will be randomized to the digital UP or one of two alternative digital psychological interventions. Data from this study will be used to develop and validate reliable selection algorithms by examining whether allocation to a predicted optimal treatment based on a short baseline questionnaire is associated with superior outcomes compared to allocation to a non-optimal treatment. Finally, the study will explore the role of user engagement, working alliance, and other factors to determine if they serve as mechanisms of treatment response. The study's results should provide guidance regarding the relative effectiveness of the UP compared to alternative leading digital interventions, determine the value of developing precision treatment rules for CBT interventions with different mechanisms of action, and further the understanding of common and treatment-specific mechanisms of change.

These and additional efforts are needed to evaluate the UP in routine clinical settings, with more heterogeneous patient populations, and under circumstances most closely representing real-world practice. Relatedly, we must continue to explore the benefits of scaling up training efforts in the effective utilization of the UP in large healthcare settings. Finally, we plan to continue developing and refine delivery methods, including digital and group applications to continue to facilitate the massive scaling up of access to evidence-based interventions required to begin to close the extensive gaps in service delivery evident for those suffering from emotional disorders.

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Received: February 1, 2024
Accepted: April 19, 2024
Available online: 29 April 2024